Greenfield Medical Centre

Department of Internal Medicine & Endocrinology
Accredited by the National Board of Healthcare Standards

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OUTPATIENT CONSULTATION REPORT

Date of Visit: March 14, 2025 | Report Generated: March 15, 2025 | Report No: GMC-2025-03847

Patient Name: Sarah Mitchell
Date of Birth: 12 June 1989 (Age: 35)
Gender: Female
NHS Number: 485 741 2390
Address: 17 Maple Grove, Leeds, LS6 3AH
Phone: 07712 334 891
GP Referral: Dr. James Hartley (Elmwood Surgery)
Consulting Physician: Dr. Priya Menon, MBBS, MRCP

1. PRESENTING COMPLAINT

The patient presents with an 8-month history of persistent fatigue, unexplained weight gain of approximately 7 kg, significant hair thinning and hair loss on the scalp, cold intolerance, constipation, and a general feeling of "brain fog" affecting her ability to concentrate at work. She reports that her menstrual cycles have become increasingly irregular over the past 5 months. The patient states that her GP initially attributed these symptoms to stress and lifestyle factors and advised rest and dietary changes without ordering any investigations.

2. HISTORY OF PRESENTING ILLNESS

- Symptoms began insidiously approximately 8 months ago following a period of significant work-related stress.
- Fatigue is described as constant and debilitating — the patient reports sleeping 10–11 hours per night and still waking unrefreshed.
- Weight gain of ~7 kg despite no significant change in diet or exercise habits.
- Hair loss is diffuse; the patient reports noticing large amounts of hair in the shower drain daily.
- Cold intolerance: patient feels cold even in warm environments; colleagues do not share the same discomfort.
- Constipation: bowel movements reduced to 2–3 times per week from daily previously.
- Cognitive difficulties: difficulty concentrating, slow processing speed, low motivation.
- Puffy face observed by the patient herself, particularly in the mornings.
- No history of neck swelling, difficulty swallowing, or voice changes noted.

3. PAST MEDICAL & SURGICAL HISTORY

- Iron-deficiency anaemia – treated in 2019, resolved with supplementation.
- Appendectomy – 2011, uncomplicated.
- No known thyroid conditions. No prior autoimmune diagnoses.

- No history of radiation therapy to the neck.

4. CURRENT MEDICATIONS & ALLERGIES

- Ferrous sulphate 200mg – PRN (occasional, self-prescribed)
- Vitamin D3 1000 IU – daily (self-prescribed)
- No known drug allergies

5. PHYSICAL EXAMINATION

- **General:** Alert, well-oriented. Appears mildly puffy in the face. Speaks slowly.
- **Vitals:** BP 102/68 mmHg | HR 58 bpm (bradycardic) | Temp 36.1°C | BMI 29.4 kg/m²
- **Skin & Hair:** Dry, coarse skin noted on hands and shins. Diffuse hair thinning on scalp. Lateral third of eyebrows sparse (possible Queen Anne's sign).
- **Thyroid:** No palpable goitre. No cervical lymphadenopathy.
- **Cardiovascular:** Regular, rate 58 bpm. No murmurs. Mild peripheral puffiness bilaterally (non-pitting oedema).
- **Neurological:** Delayed relaxation phase of ankle reflexes noted (characteristic finding).
- **Abdomen:** Soft, mildly distended. No organomegaly.

6. INVESTIGATIONS & LABORATORY RESULTS

Test	Result	Reference Range	Status
TSH (Thyroid Stimulating Hormone)	**11.4 mIU/L**	0.4 – 4.0 mIU/L	**HIGH ↑**
Free T4 (Thyroxine)	**7.2 pmol/L**	12.0 – 22.0 pmol/L	**LOW ↓**
Free T3 (Triiodothyronine)	**2.9 pmol/L**	3.1 – 6.8 pmol/L	**LOW ↓**
Anti-TPO Antibodies	**342 IU/mL**	< 34 IU/mL	**HIGH ↑ (Autoimmune)**
Anti-Thyroglobulin Antibodies	**198 IU/mL**	< 115 IU/mL	**HIGH ↑**
Full Blood Count – Haemoglobin	**10.9 g/dL**	12.0 – 16.0 g/dL	**LOW ↓ (Mild anaemia)**
Serum Ferritin	**9.2 ng/mL**	12 – 150 ng/mL	**LOW ↓**
Serum Cholesterol (Total)	**6.8 mmol/L**	< 5.0 mmol/L	**HIGH ↑**
Blood Glucose (Fasting)	5.1 mmol/L	3.9 – 5.5 mmol/L	Normal
Liver Function Tests	Within limits	—	Normal
Renal Function (eGFR)	78 mL/min	> 60 mL/min	Normal
Vitamin D (25-OH)	**18 nmol/L**	50 – 125 nmol/L	**DEFICIENT ↓**
Vitamin B12	**168 pg/mL**	200 – 900 pg/mL	**LOW ↓**
Thyroid Ultrasound	Mildly heterogeneous echogenicity. No nodules. Gland slightly reduced in volume. Consistent with early		**Abnormal**

Test	Result	Reference Range	Status
	Hashimoto's thyroiditis.		

7. ASSESSMENT & DIAGNOSIS

Primary Diagnosis: Hypothyroidism secondary to Hashimoto's Thyroiditis (Autoimmune Thyroiditis)

The clinical presentation, elevated TSH, suppressed free T4 and T3, markedly elevated Anti-TPO and Anti-Thyroglobulin antibodies, and ultrasound findings are all consistent with a diagnosis of Hashimoto's Thyroiditis — an autoimmune condition in which the immune system attacks the thyroid gland, reducing its ability to produce sufficient thyroid hormone.

Secondary Findings:

- Iron-deficiency anaemia (recurrent) — likely exacerbated by hypothyroid state
- Vitamin D deficiency — requires therapeutic supplementation
- Vitamin B12 borderline low — to be monitored
- Secondary hypercholesterolaemia — expected to improve with thyroid treatment

8. MANAGEMENT PLAN

- **Levothyroxine 50mcg** daily, to be titrated. Commence immediately.
- Thyroid function repeat in **6–8 weeks** to reassess TSH and Free T4.
- **Ferrous fumarate 210mg** twice daily for iron-deficiency anaemia.
- **Cholecalciferol 20,000 IU** weekly for 8 weeks, then reassess Vitamin D levels.
- **Vitamin B12** supplementation initiated (1000mcg daily oral).
- Dietary advice: high-fibre diet for constipation; avoid excessive iodine or raw cruciferous vegetables.
- Patient counselled on the chronic nature of Hashimoto's thyroiditis and the expected timeline for symptom improvement (6–12 weeks after starting treatment).
- Referral to dietitian for weight management support.
- Follow-up appointment in **8 weeks**. If TSH remains elevated, increase Levothyroxine to 75mcg.

9. CLINICIAN'S NOTES

Patient was understandably frustrated that these symptoms — which had significantly impacted her quality of life for over 8 months — were not investigated by her GP despite multiple presentations. She should have been referred for thyroid function tests at her second or third visit. The elevated antibodies suggest this condition has likely been developing over a prolonged period. Prognosis with appropriate thyroid hormone replacement is excellent. Patient is aware.

Dr. Priya Menon MBBS, MRCP
Consultant Endocrinologist
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Electronically signed: 15 March 2025
Verified & validated ✓

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